INVESTIGATIONS

NURSES NOTES

Sticker

TO SEND / COLLECT							
	Hb: 9.5 (2/2/26)	Platelet Counts:	Urine R/M:	O2		CHECK RYLE'S TUBE POSITION BEFORE EACH FEED	CHARGES
	TLC:	DLC:	CXR:	Monitor			LAB
	B. Sugar:	Na/K: 134.3/3.36		ECG			
	Urea:	Creatinine: 0.68	MISC:	Pace Maker			
	CPK:	MB:		Nebulizer			
	S. Bil T/D/I:			Infusion Pump			
	SGOT / PT / ALP:			B. Sugar			OTHERS
	S. Proteins/Alb/Glob:			Dialysis			
	PTT/PT/INR:			Ventilator / Bipap			

	DOSE	FREQ	ROUTE	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23	24	1	2	3	4	5	6	7	ORDERS
VENT MODE				RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	
RATE / FIO2				–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	
CVP / PAP				–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	–	
SpO2				98%	98%	98%	98%	97%	98%	98%	97%	98%	98%	98%	98%	98%	98%	98%	98%	98%	98%	98%	98%	98%	98%	98%	98%	
PULSE / HR (min)				80	80	80	80	80	80	80	80	80	105	80	80	80	80	80	80	80	80	80	80	80	80	80	80	
BP				120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80	120/80	
TEMP				98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	98.4°F	
RR				18/min	20/min	20/min	20/min	20/min	20/min	20/min	20/min	18/min	20/min	20/min	20/min	20/min	20/min	20/min	20/min	20/min	20/min	20/min	20/min	20/min	20/min	20/min	20/min	
RBS																												
HEMATOMA																												
DISTAL PULSES																												
INTAKE IV				1000 ml	1000 ml	1000 ml	1000 ml									1000 ml	1000 ml	1000 ml	1000 ml	1000 ml							1000 ml	2100 ml / 1500 ml
INTAKE ORAL				200 ml		100 ml				200 ml	100 ml					200 ml		200 ml										
INTAKE R.T.																												
OUTPUT URINE				200 ml		150					300 ml							200 ml				300 ml		400 ml				
OUTPUT R.T.																												
OUTPUT STOOL																												
OUTPUT DRAIN																												

[redacted]

STATUS : CASH / MEDICLAIM / GOVT. PANEL

WARD PATIENT'S DAILY RECORD SHEET

CONSULTANT'S NAME [redacted]

PATIENT'S NAME [redacted]

DAY 5th / 6th

HB/SC 20/F ROOM HDW IPD NO. 1454 UHID 25/2026

START DATE	S.No.	DESCRIPTION DRUGS	ORDERS DOSE	FREQUENCY	ROUTE	ADMINISTRATION TIME DATE 8/2/26	DATE 9/2/2026
	1	INJ - SUPACEF (DS)	1.5gm	TWICE	IV		
	2	INJ - PAN	40mg	TWICE	IV		
	3	INJ - EMEST	4mg	TWICE	IV		
	4	INJ - TRAMADOL	50mg	TWICE	IV		
	5	INJ - OFLOX (DS)	200mg	TWICE	IV		
	6	INJ - PCM	1gm	TWICE	IV		
	7	INJ - PERINORM		THRICE	IV		
	8	INJ - CLEXANE	0.6ml	1D	S/C		
	9						
	10						
	11	T - THYRONORM	25mcg	ONCE	P/O		
	12	CAP - BIFILAC	1cap	TWICE	P/O		
	13	CAP - ZEDOTT	1cap	OD	P/O		
	14	CAP - ZERODOL	1cap	OD	P/O		
	15						
	16	NEB - DUOLIN	1ampule	THRICE	P/N		
	17	NEB - BUDECORT	1ampule	THRICE	P/N		
	18	SYP PETROLOX	10ml	TID	P/O		
	19	INJ - HYDROCORT	100mg	STAT	I/V		
	20	INJ - AVIL	1amp	STAT	I/V		
	21	INJ - LASIX	1amp		I/V		
	22						
	23						
	24						
	25						
	26						
	27						
	28						
	29						

[redacted]

DATE 8/2/26	IV REGIME & TIMINGS 9/2/2026 DATE
IV Fluid NS 40ml/hr	IV Fluid NS 40ml/hr

INVESTIGATION TO SEND/ COLLECT DATE 8/2/26	DATE 9/2/2026	PROCEDURES
CBC sample done at 8am		
NC Arranged		
		DIET Soft Diet High Protein diet.

NURSE NOTES	
DATE	
O2	
ECG	
NEBULISER	
BUN	
B. SUGAR	
INFUSION PUMP	
DIASS	
DIASS	

PL. CHECK R.T POSITION BEFORE EVERY FEED

NAME & NATURE OF SURGERY & ANY STAT MEDICATIONS

Medicine not due.
1 unit Blood Start at 12:20AM and Stop 4:00AM

Chart Fluids, Monitors, I/V Lines, O2 % Hourly Regularly

Total Intake/Output 3500 / 1500 Balance RTA Drain

HSB/HIV/HCV N/A Drug Allergy NIL Bedsore DVT Prophylaxis

DATE 8/2/26	INVESTIGATION	DATE 9/2/2026

[redacted]

DAILY FLUID BALANCE CHART

DAY & DATE

HOUR ENDING	ORAL AMOUNT	ORAL TYPE	INTRAVENOUS PUT UP	INTRAVENOUS GIVEN	URINE	ASPIRATION or VOMIT
7am	100ml	H2o	100ml	often		
	100ml	Tea	100ml	Pen		
9am	200ml	Porridge				
	100ml	H2o	100ml	Sofrac		
			100ml	Tramadol		
10am	50ml	Fruit				
12pm	100ml	H2o				
2pm	250ml	water	100ml	Pen	1000ml	
3pm	50ml	Tea				
5pm	50ml	Soup				
8pm	200ml	Dinner	100ml	after		
	100ml	H2o	100	Sofrac		
			100	Rytmin	1000ml	
			100	Tramadol		
24 HOURS TOTAL			2200		2000	

9/2/2026

DAY & DATE

HOUR ENDING	ORAL AMOUNT	ORAL TYPE	INTRAVENOUS PUT UP	INTRAVENOUS GIVEN	URINE	ASPIRATION or VOMIT
7AM	100ml	Tea	100ml	PCM		
	100ml	H2O	100ml	Oflox		
9AM	[illegible]		[illegible]	[illegible]	[illegible]	
	[illegible]	H2O	[illegible]	Pen		
	[illegible]	Tea	[illegible]	[illegible]		
10AM	[illegible]	[illegible]	[illegible]	[illegible]		
1PM	[illegible]	[illegible]	[illegible]	[illegible]	[illegible]	
	[illegible]	[illegible]	[illegible]	Pen		
4PM	[illegible]					
	[illegible]	H2O				
5PM	[illegible]	Soup				
8PM	[illegible]				1000ml	
	[illegible]					
24 HOUR TOTAL	Total 2 [illegible]				Total 1600 [illegible]	

SISTER ON DUTY

[redacted]

COMA CHART

| OBSERVATIONS EVERY HOUR | | | | |

[redacted]

STATUS : CASH / MEDICLAIM / GOVT. PANEL

WARD PATIENT'S DAILY RECORD SHEET

CONSULTANT'S NAME: [redacted]

PATIENT'S NAME: [redacted]

AGE/SEX: [illegible] IPD NO: [illegible] UHID: [illegible]

DAY 7th /

START DATE	S.No.	DESCRIPTION DRUGS	ORDERS DOSE	FREQUENCY	ROUTE	ADMINISTRATION TIME DATE 10/2/26				DATE			
	1	INJ SUPACEF	1.5GM	TWICE	IV				6pm				
	2	INJ PAN	40mg	TWICE	IV	8am		8pm					
	3	INJ EMSET	10mg	THRICE	IV	8am	6am	2pm					
	4	INJ TRAMADOL	50mg	TWICE	IV	8am			6pm				
	5	INJ OFLOX	200mg	TWICE	IV	8am		8pm					
	6	INJ PCM	1GM	THRICE	IV	8am	2pm	8pm					
	7	INJ PERINORM		THRICE	IV	8am	2pm		10pm				
	8	INJ CLEXANE	0.6ml	1.0	S/C				10pm				
	9												
	10	TAB THYRONORM	50mcg	ONCE	P/O	8am							
	11	CAP BIFILAC	1cap	THICE	P/O	8am			10pm				
	12	CAP ZEDOTS	1cap	SOS	P/O								
	13	CAP ZEPODIUM	1cap	SOS	P/O								
	14												
	15	NEB-DUOLIN	1vial	THRICE	P/N	8am	2pm	8pm					
	16	NEB BUDECORT	1vial	TWICE	P/N	8am	8pm						
	17	SAP PATRICE	1vial	THRICE	P/N	8am	2pm	8pm					
	18												
	19												
	20												
	21												
	22												
	23												
	24												
	25												
	26												
	27												
	28												
	29												

[redacted]

DATE	IV REGIME & TIMINGS	DATE
10/2/26	IV 5%DNS 125 ml/hour I.V	

INVESTIGATION TO SEND/ COLLECT

DATE 10/2/26	DATE	PROCEDURES
[redacted]		
		DIET Soft Diet High Protein diet

NURSE NOTES			
DATE			
O2			
ECG			
NEBULIZER			
BP/PR			
B. SUGAR			
INFUSION PUMP			
DIURESIS			
DIAGNOSIS			

PL. CHECK R.T POSITION BEFORE EVERY FEED

NAME & NATURE OF SURGERY & ANY STAT MEDICATIONS

Check I/V Fluids, Monitors, I/V Lines, O2 ½ Hourly Regularly

Total Intake/Output: Balance: RTA: Yes Drain: No

HSBAg/HIV/HCV: N.K. Drug Allergy: N.K. Bedsore: Yes DVT Prophylaxis: No

DATE	INVESTIGATION	DATE
10/2/26		

[redacted]

DAILY FLUID BALANCE CHART

DAY & DATE

10/2/25

HOUR ENDING	ORAL AMOUNT	ORAL TYPE	INTRAVENOUS PUT UP	INTRAVENOUS GIVEN	URINE	ASPIRATION or VOMIT
7 AM	100ML	TEA			900	
	10/0ML	H2O				
9AM	100ML	B.P				
	50ml	Tea	100ml	Sucrose		
10AM	100	H2O	100ml	Dextrose		
24 Hours TOTAL						

TEMPERATURE

PULSE

RESPIRATION

	6	8	10	12
	A.M.	A.M.	A.M.	A.M.

SISTER ON DUTY

	Dr.	Dr.
M	[redacted]	
A/N		
N		

DAY & DATE

HOUR ENDING	ORAL AMOUNT	ORAL TYPE	INTRAVENOUS PUT UP	INTRAVENOUS GIVEN	URINE	ASPIRATION or VOMIT
24 Hours TOTAL						

COMA CHART

OBSERVATIONS EVERY _____ HOUR _____ Date _____

TIME	10/2/25				
LEVELS OF CONSCIOUSNESS	Alert & Awake	✓			
	Drowsy & Sl Confused	✓			
	Very Drowsy, Reacts to word	✓			
	Semi-comatose Reacts to pain	✓			
	Unresponsive	✓			
PUPILS	Size & R.T.				
	Reaction				
	Size & R.T.				
	Reaction				

BLOOD PRESSURE (GREEN)

PULSE (RED)

RESPIRATION

| | | | | | |

[redacted]

2

ICU-10

INFORMED CONSENT FOR SURGERY / PROCEDURE

INFORMED CONSENT FOR Rt TKR

I, [redacted] ☐ the patient, or ☐ (representative of patient .....). (please tick the correct option above and below).

I have read and have been explained this consent form in English (name of language) which fully I understand and understood the information provided about ..... (full name of operation/procedure) given below in this consent form.

B.2. description of operation/procedure.....

Intended benefits

1. Relief from diagnosed illness
2. _____
3. _____
4. _____
5. _____
6. _____

I understand that all procedures carry risks. The potential risks and complications from this procedure are:

1. Bleeding
2. Infection
3. _____
4. _____
5. _____
6. _____
7. _____
8. _____
9. _____
10. _____

I have been explained the implications of not undergoing this procedure and the alternative metho[redacted]f treatment like

1. _____
2. _____
3. _____
4. _____

I am now aware of the intended benefits, possible risks & complications, and av[redacted]e the said operation/ procedure. I am also aware that results of any operation/procedure can va[redacted]nt; and I declare that no guarantees have been made to me regarding success of this operation/procedur[redacted] majority of patients have an uneventful operation and recovery, few cases may be associated with complicatio[redacted] the common risks and complications associated with this operation/procedure as listed in Part B, and understand that it i[redacted]ssible to list all possible risks and complications of any operation/procedure.

I also understand that sometimes a planned operation/procedure may to be postponed or cancelled if patient's clinical condition worsens or due to any unforeseen technical reason. I am also aware that I can withdraw my consent at any point of time at my own risk and consequence, by submitting the withdrawal in writing.

Initials of Patient / Patient's Representative (only if Patient is a minor or unable to give consent)_____

I understand that if medical exigencies demand, further or alternative operation/procedure measures may need to be carried out, and in such case there may be difference in the planned and actual operation/procedure. I am aware that I may require administration of blood and/or blood products during or after the operation/procedure as found necessary by the doctor (for which a separate consent shall be obtained).

I am now also aware that during the course of this operation/procedure the doctor will be assisted by medical and paramedical team, and that the doctor may seek consultation/assistance from relevant specialists if the need arises.

I agree to observing, photography (still/video/television) of the procedure (including relevant portions of my body) including my diagnosis/report (pathology, radiology, etc), for academic/medical/medico-legal purposes, provided my identity is not relevant by such acts. I also agree to my clinical details being shared for scientific publications if my identity is not disclosed.

I am also aware of the expected course after the operation/procedure and the care to be provided, and understand that sometimes admission to an intensive care unit and/or extension of duration of hospitalization may be required and/or there may be requirement of extra medicines or treatments, therapy leading to increase in the treatment expenses, depending upon the body's response to the treatment/procedure.

I authorize the hospitals for disposal of any tissue or body part that may be removed from my body in the appropriate manner during and for the purpose of conducting this operation/procedure.

I declare that I have received & fully understood the information provided in this consent form, that I have been given an opportu[redacted] to ask questions relating to my aliment, the operation/procedure being performed, its risks, consequences, alternative, potential complications and intended benefits and recovery, and that all my question have been answered to my entire satisfaction and there are no misconceptions or false hopes in my mind. I further declare that all the fields (of this form) requiring insertion or completion were filled in my presence at the time of my signing this form.

For the above mentioned operation(s)/procedures(s) that I have been made aware of, I give my consent voluntarily to Dr. _____ [redacted] _____(name of the doctor performing the operation/procedure) or carrying out the said operation/procedure on myself or my above named patient being fully aware of the nature, potential risks and complications, intended benefits and possible alternatives.

I, the above named Patient/ named patient's representatives, do further hereby declare that I am above 18 years of age as on the date of signing this form, mentally sound and am giving consent without any fear, theat of false misconception.

	Signatu[redacted]	Name	Date	Time
Patient [redacted]	[redacted]	[redacted]	5/2/25	8 Am.
Relatives/Guardian	[redacted]	Grandson (Write name & relationship with Patient)	5/2/25	8 Am.
Witness				

* Right Hand for Males & Left Hand for Females # only if patient is a minor or unable to give consent

I, the undersigned doctor, have explained the [redacted]al risks and complications, intended benefits, expected post-procedure course, and possible alternative[redacted] operation/procedure, to the patient/patient's representative. I am confident that he/she has understood t[redacted]ully as described in this document.

Consent obtained by:

Dr. [redacted] _____ Des[redacted]n & Dept. DOD _____ Signature: [redacted]

Date:_____ Time:_____

Does this patient/procedure require an additional/ HIGH RISK consent? YES NO [redacted]

Doctor performing the procedure : Dr._____ Dept._____ Signature:_____

For Internal/Hospital use only

[redacted]

INFORMED CONSENT FOR ANESTHESIA

I, [redacted] ☐ the patient, or ☐ (representative of patient .....), have (please tick the correct option above and below).

I have read and have been explained this consent form in English (name of language) which I fully understand, that I would require 'Anesthesiology Services' for getting ..... (Operation/Procedure) performed.

It has been explained to me that all forms of anesthesia involve some risks, and though rare, anesthesia related complications like drugs reactions, infections, bleeding, blood clots, loss of sensation, loss of limb function, paralysis, stroke, brain damage, heart attack and death can occur, I understand that these risks apply to all forms of anesthesia and that additional risks specific for the type anesthesia as applicable in my case and marked below have been explained to me as well. I understand that the type(s) for anesthesia service checked below will be used for my procedure and that the anesthetic technique to be used is d[redacted]y facto including my physical condition, the type of procedure my doctor is to perform, anesthe[redacted]quirement of the procedure and accommodating my preference to the extent feasible. It [redacted]mes, an anesthesia technique particularly involving the use of local anesthetics, with or [redacted]y fail to produce desired effect therefore me have to so supplemented with another technique including adminis[redacted] general anesthesia.

• General Anestheisa	Technique	Drug injected into the bloodstream, breathed into the lungs, or by other routes.
	Expected result	Total unconscious state, possible placement of a tube into the windpipe.
	Major Risks	Mouth or threat pain, hoarseness, injury to mouth or teeth, awareness under anesthesia, injury to blood vessels, aspiration, pneumonia, convulsions, Pneumothorax.
• Spinal or Epidural Analgesia • With sedation • Without sedation	Technique	Drug injected into the bloodstream, breathed into the lungs, or by other routes.
	Expected result	Total unconscious state, possible placement of a tube into the windpipe.
	Major Risks	Mouth or threat pain, hoarseness, injury to mouth or teeth, awareness under anesthesia, injury to blood vessels, aspiration, pneumonia, convulsions, Pneumothorax.
• Nerve Block • With Sedation • Without Sedation	Technique	Drug injected into the bloodstream, breathed into the lungs, or by other routes.
	Expected result	Total unconscious state, possible placement of a tube into the windpipe.
	Major Risks	Mouth or threat pain, hoarseness, injury to mouth or teeth, awareness under anesthesia, injury to blood vessels, aspiration, pneumonia, convulsions, Pneumothorax.
• Intravenous Regional Anesthesia • With Sedation • Without Sedation	Technique	Drug injected into the bloodstream, breathed into the lungs, or by other routes.
	Expected result	Total unconscious state, possible placement of a tube into the windpipe.
	Major Risks	Mouth or threat pain, hoarseness, injury to mouth or teeth, awareness under anesthesia, injury to blood vessels, aspiration, pneumonia, convulsions, Pneumothorax.

• Monitored Anesthesia Care with sedation	Technique	Drug injected into the bloodstream, breathed into the lungs, or by other routes producing a semiconscious state.
	Expected result	Reduced anxiety and pain, partal or total amnesia
	Major Risks	An unconscious state, depressed breathing (requiring ventilation) injury to blow vessels
• Monitored Anesthesia Care without sedation	Technique	None
	Expected result	Measurement of vital signs, availability of anesthesia provider for interventions as deemed fit.
	Major Risks	Awareness, anxiety and/ or procedure related discomfort.
• Rectal/Vaginal/Derma Medication	Technique	Medicine introduced through Rectum/Vagina/Skin to produce loss of sensation
	Expected result	Producing loss of sedation in patients who can't take oral/IV/IM medication or in whom injections are to be avoided.
	Major Risks	Discomfort
• Vascular Cannulation	Technique	Cannulation of artery or large veins
	Expected result	Measurement of invasive arterial pressures/fill up pressures.
	Major Risks	Haematoma, bleeding, ischaemic related complications.

BY SIGNING THIS FORM, I GIVE MY FULL, FREE, VOLUNTARY CONSENT WITHOUT ANY RESERVATION TO DR.

[redacted].....(name of doctor administering anesthesia) or his/her associated, for administering anesthesia to [ ] MYSELF or [ ] MY PATIENT [redacted].....(name of patient), being fully aware of the nature, risks and complications.

Signed on...../...../ at .....Am/Pm

		Name
Patient [redacted]	[redacted]	[redacted]
Relatives/Guardian		Friend Son (Write name & relationship with Patient)
Witness		
Interpreter (If applicable)		

only if patient is a minor or unable to give consent

I, the undersigned Doctor, have e[redacted]re, potential risks and complications, expected benefits, expected post-anesthesia recovery, and po[redacted]ive to the patient/patient representative. I am confident that he/she have understood the information pr[redacted]n this documents.

Consent obtained by

Dr. [redacted]..... Designation & Dept. ....DoD Anaesthesia..... Signature..[redacted].....

*(being person administering/Anesthsia or member of his/her team)

For Internal/Hospital use only

A9H/IPD/2025-26/INF-CON-ANES

[redacted]

Patient Name:.....UHID No.:.....IPD No.:.....

Age/Sex:.....Bed No:.....D.O.A.....

Proposed Date & Time of Surgery: Weight: Height :-

PRE ANESTHETIC CHECKUP & ANESTHESIA RECORD FORM

HISTORY

1. Present History-cough/breathlessness/ chest Pain, Palpitations of Blackouts/ Convulsions of Fits/ Bruising or bleeding problems/ Any other serious illnesses.
2. Past History - T.B / Hypertension/ Diabetes/ Heart attack or heart failure / jaundice.
3. Drug History any medicine or drugs intake (including inhalers, eye drops, creams. Or herbal remedies allergy to any drugs or materials/ women-pregnancy/ pill or HRT
4. Personal History- smoke/ alcohol/ other addition
5. Past surgery / Anesthesia History any previous operations or anesthetics/ any problems with anesthetics with You or family.

[redacted]

1. [redacted] D.yrs
2. [redacted] 1 oD
3. .....
4. .....

EXAMINATION

Pallor —	Pulse 78	CVS	Oral Hygiene
Icterus / Jaundice	B. P. 130/78	RS	Dentition- false, capped or crowned
Cyanosis —	Respiration NBS (+)	ABD NAD	Teeth Edentulous
Pedal Oedema —	Temperature (N)	CNS	Mouth Opening >2 finger
Lymphadenopathy —		SPINE	Neck
		FUNDUS EXAM	Airway

Plan of Anaesthesia:- ↓ CSE

INVESTIGATIONS – WNL

HB	B. SUGAR-F	Urine Exam	X-Ray
PCV	PP	LFT:	ECG
TLC	R		Echo
DLC	S. Na+		Hb5Ag
ESR	S. K+		HIV
AEC	S. Ca ++		
PLATELETS	B. Urea:		
BT: ☐ CT: ☐ PT: ☐	S. Creatinine:		

USGASA Grading- I / (II) / III / IV / V / VI / E

Advice : FIT/UNFIT For Surgery under Anesthesia

Any Investigation required :

Treatment required

Pre Anesthetic Orders:

1. Nil orally From: 0 AM
2. Written consent to be taken
3. Arrange unit of crossed matched blood 6 PRBC
4. Premedications Night before surgery
5. Shift Patient to OT By .....

[redacted]

[redacted]

Name & Signature of Anesthetist

Time:_____(AM/PM)

[redacted]

NAME: AGE/SEX:

BED NO: UHID NO.:

TREATING CONSULTANT:

ANESTHESIA MONITORING SHEET

PER - Operative Records

Surgical Procedure:-

Anesthesiologist: [redacted]

Surgeon:-

Anesthesia Time 6:40 pm Operation Time 7:05 pm Completion Time 9:10 pm

Anesthesia Technique
GA
Regional Anesthesia
Local Anesthesia
MAC
Inducing Agents-
Muscle Relaxants-
Anesthetic Agents-

CSE
Epidural
SAB

1 APP | Epidural catheter placed at L3-L4 | Midline | Sitting | 18G Tuohy needle | Test dose given
1 APP | SAB at L4-L5 | Midline | Sitting | 26G Quincke |
0.5% 3.4 ml (4) Bupivacain + 25mcg fentanyl |

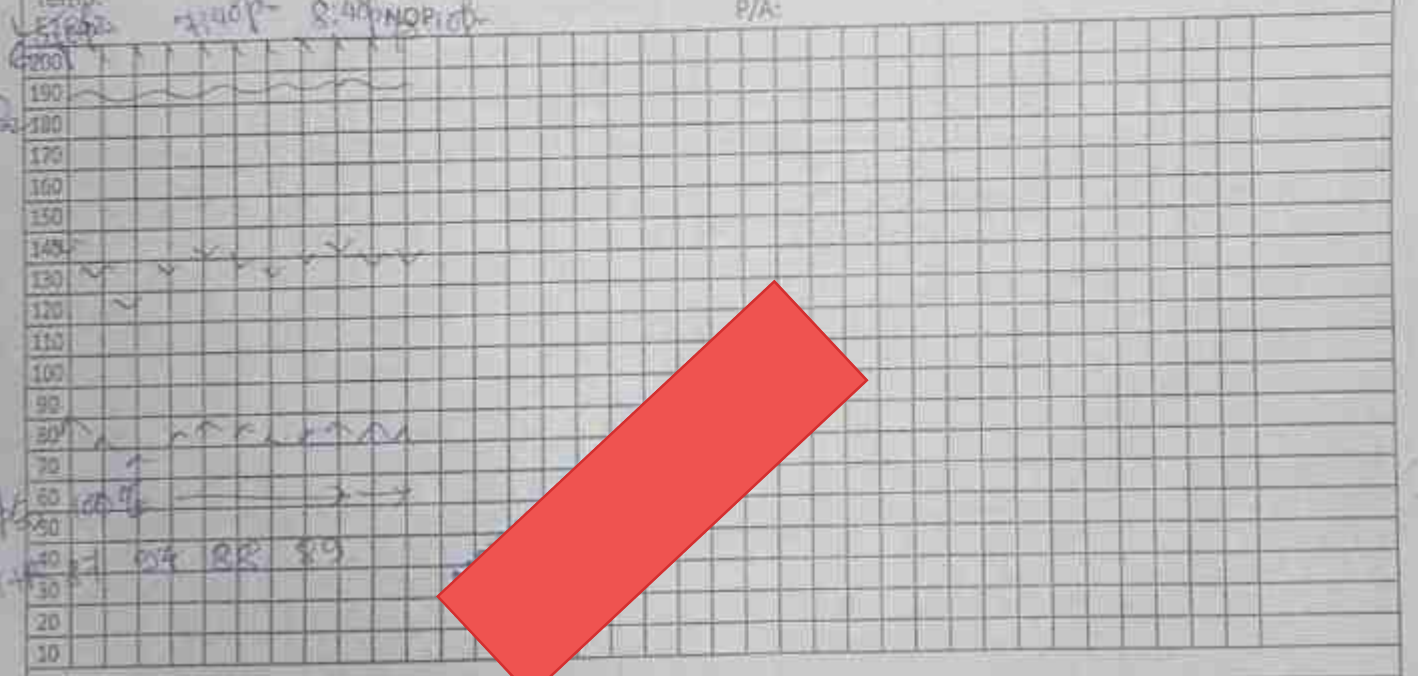
Immediate Pre-anesthesia Revaluation:-

ECG:
NIBP:
Temp:
ETCO2:

SPO2:
IBP:
CVP:
NPO: 8:40 pm

CVS:
RS:
CNS:
P/A:

AIRWAY:



[redacted]

Intra op IV Fluids NS → NS
Blood
Blood Loss 400 ml
Urine Output 200 ml

Reversal Neostigmine
Atropine Glycopyrrolate

Time: AM/PM
Drugs -

Recovery Score

Post Operative Condition (Recovery Room)

Pulse 79
B.P. 110/70
Temperature
RR
Color
Sensorium
Spo2 98% in Room

Shift to Ward/ICU
NPOTI
Monitor
Analgesia Section
IV Fluids

Name & Signature with Employee ID of Nurse
Time [redacted] 6:30 pm (AM/PM)

Name & Signature of Anesthetist
Time [redacted] 6:40 pm (AM/PM)

Name & Signature of Chief Surgeon
Time 9:10 [redacted] (AM/PM)

[redacted]

For Internal/Hospital use only

[redacted]

[redacted]

[redacted]

F/SEX: .....

SURGICAL SAFETY CHECKLIST (WHO)

BEFORE INDUCTION OF ANAESTHESIA
(with at least nurse and anaesthesia)

Has the patient confirmed his/her identity, sit, procedure and consent?
- [x] Yes

Is the Site Marked?
- [x] Yes
- [ ] Not Applicable

Is the Anaesthesia Machine & Medication Check complete?
- [x] Yes

Is the Pulse Oximeter On the Patient Functioning?
- [ ] No
- [x] Yes

Does patient have a?

Known Allergy
- [x] No
- [ ] Yes

Difficult Airway/aspiration Risk
- [ ] No
- [x] Yes

Risk Of > 500ml Blood Loss (7ml/kg In Children)
- [ ] No
- [x] Yes

[redacted]

Name & Sign of Nurse
6:30 pm

For Internal/Hospital use only

BEFORE SKIN INCISION
(with nurse, anaesthetist and surgeon)

- [x] Confirm All Team Members Have Introduced Themselves By Name And Role?
- [x] Confirm the patient's name, procedure and where the incision will be made?

Has Antibiotic Prophylaxis Been Given Within The Last 60 Minutes?
- [ ] No
- [x] Yes

Anticipated Critical Events

To Surgeon
- What Are The Critical Steps?
- How long will the case take?
- What is the anticipated blood loss?

To Anaesthetist
- [ ] Are there any patient specific concerns?

To Nursing Team
- [x] Has Sterility Been Confirmed?
- [ ] Are There Equipment Issues?

Is Essential Imaging Displayed?
- [ ] No
- [x] Yes

[redacted]

Name & Sign of Surgeon
9:20pm

BEFORE PATIENT LEAVES OPERATING ROOM
(with nurse, anaesthetist and surgeon)

Nurse Verbally Conforms
- [x] Name of the procedure
- [x] Completion of instrument, sponge and needle
- [ ] Specimen labelling
- [ ] Whether there are any equipment problems to be addressed?

To Surgeon, Anaesthetist & Nurse
- [ ] What are the key concerns for recovery and management of this patient?

[redacted]

[redacted]

Name & S... 6:40pm

[redacted]

[redacted]

[redacted]

[redacted]

15/2/26

IPD No. [redacted]

OPERATION THEATRE NOTES:

Surgeon In-charge: [redacted] Anaesthetist: [redacted]

Assistant Surgeon: OT Technician: [redacted]

OT Staff: [redacted] Type of Anaesthesia: ✓ CSE

Pre-Operative Diagnosis: Osteoarthritis Bilateral knee

Post-Operative Diagnosis:

Procedure Name: Total knee Replacement Right side

Operation Started at: 7:00 pm Operation Finished at: 9 pm Duration: 2 hrs

Sponge Count: 20 Whom: Dhiraj

Operative Notes:

After adequate effect of the anaesthesia painting and draping of the part was done. Tourniquet inflated. Medial parapatellar approach used to expose the knee joint using midvastus. Tibial and femoral cuts taken. Trial implant found to be satisfactory. B femur, 1 Tibia. Insert 9 m size 10.5 x 100 final implant placed. Closure done in layers. Drain. Patient shifted to the post operative ward in stable vital condition.

Organ Explained: NA

Specimen Sent for histopathology (if any): Nil

Immediate post-operative condition: [redacted]

Surgeon's Signature: [redacted]

Date & Time: 15/2/26 9:10 (am/pm)

For Internal/Hospital use only

A9H/IPD/2025-26/OT NOTES

Post Operative Notes:

- NPO for 6 hrs
- Inform surgeon
- Monitor vitals
- O2 by face mask 6 lt/min SaO2
- RL/NS - 60 ml/hr
- Epidural infusion
  - 20 ml 0.1% Ropivacaine + 2 ml fentanyl + 28 ml NS
  - 4-6 ml/hr
- Transfuse 10 PRBC

Mean

[redacted]

Signature:

Name: Dr. Sreelekha, Dr. Geetha

Time:

- Carry normally

Recovery Room Notes:

- Oxygen @ 4 lt/min for 12 hrs
- Transfuse 10 PRBC
- IV fluid NS @ 80 ml/hr
- Inj Sulperazone 1.5 gm IV BD
- Inj Ofloxacin 200 mg IV BD
- Inj Pantop 40 mg IV BD
- Inj Emeset 4 mg IV TDS
- Inj Tramadol 50 mg IV TDS
- Inj Paracetamol 1 gm IV TDS
- Monitor vitals
- Inj Clexane 0.6 ml SC
- DVT Pump

Xray Chest B/L lower zone

Signature:

Name: Dr. Anil Kumar Garg

Time:

- Limb Elevation + Active foot movement

ID: 26320

5/2/26

LOT C2511005 | REF MK-MLPSKE109

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POSTERIOR STABILIZED
TIBIAL ARTICULAR SURFACE
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IN 2025-11-16 | 2030-11-15

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Rev.02
MH/AW/KS/010

LOT A2504222 | REF MK-UFPSRB00

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SIZE B RIGHT

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IN 2025-11-14 | 2030-11-13

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Meril Healthcare Pvt. Ltd.
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Rev.02
MH/AW/KS/010

LOT U29162714 | REF RTUUX100-RK

FREEDOM® TOTAL KNEE SYSTEM
STEMMED TIBIAL BASE PLATE
SIZE 1

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2025-10-01 | 2030-09-30

Maxx Orthopedics
2460 General Armistead Ave.
Suite 100, Norristown
PA 19403, U.S.A.
maxx Orthopedics | CE 2460 | STERILE EO
Rev.03

LOT U29202707 | REF RSUU3105-RK

FREEDOM® TOTAL KNEE SYSTEM
STEM EXTENSION
SIZE 10.5MM x 100MM

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(240)RSUU3105-RK

2025-09-01 | 2030-08-31

Maxx Orthopedics
2460 General Armistead Ave.
Suite 100, Norristown
PA 19403, U.S.A.
maxx Orthopedics | CE 2460 | STERILE R
Rev.03

Attach to patient's record
68057851 PALACOS® R
LOT 77421505
2030-08-31
Heraeus Medical GmbH, 61273 Wehrheim, Germany

[redacted]

[redacted]

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[redacted]

IP NO: 1454

Visit ID	: **RCI25006**	Registration	: 10/Feb/2026 11:34AM
UHID/MR No	: RCL0000022214	Collected	: 10/Feb/2026 11:34AM
Patient Name	: [redacted] **IPD**	Received	: 10/Feb/2026 11:35AM
Age/Gender	: 75 Y 1 M 0 D /F	Reported	: 10/Feb/2026 12:56PM
Ref Doctor	: [redacted]	Status	: Final Report
Client Name	: [redacted]	Client Code	: 315
Client Add	:	Barcode No	: 10083247

DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Range	Method
LIVER FUNCTION TEST				
Sample Type : SERUM				
TOTAL BILIRUBIN	1.07	mg/dl	0.1-1.2	Diazotized, Sulfanilic
CONJUGATED (D. Bilirubin)	**0.32**	mg/dl	<0.3	Jendrassik & Groff
UNCONJUGATED (I.D. Bilirubin)	0.75			
SGOT	**61**	U/L	<31	Enzymatic,IFFC
S.G.P.T	28	U/L	0-34	Enzymatic,IFFC
ALKALINE PHOSPHATASE	94	U/L	30-120	Kinetic, IFCC
TOTAL PROTEINS	**5.91**	gm/dl	6.6-8.3	Biuret
ALBUMIN	**2.97**	gm/dl	3.4-4.8	BCG
GLOBULIN	2.94	gm/dl	2.0-4.1	Calculated
A/G RATIO	1.01		1.0-2.0	Calculated
GGT	**53**	U/L	<38	Colorimetric Method

Note- 1. In an asymptomatic patient, Non alcoholic fatty liver disease (NAFLD) is the most common cause of increased AST, ALT levels. NAFLD is considered as hepatic manifestation of metabolic syndrome.

2. In most type of liver disease, ALT activity is higher than that of AST; exception may be seen in Alcoholic Hepatitis, Hepatic Cirrhosis, and Liver neoplasia. In a patient with Chronic liver disease, AST:ALT ratio>1 is highly suggestive of advanced liver fibrosis.

3. In known cases of Chronic Liver disease due to Viral Hepatitis B & C, Alcoholic liver disease or NAFLD, Enhanced liver fibrosis (ELF) test may be used to evaluate liver fibrosis.

4. In a patient with Chronic Liver disease, AFP and Des-gamma carboxyprothrombin (DCP)/PIVKA II can be used to assess risk for development of Hepatocellular Carcinoma.

[redacted]

MBBS,DCP
Consultant Pathologist

[redacted]

Clinical correlation is essential for final diagnosis | If test result are unsatisfactory please contact personally or on phone. This report is for perusal of doctors only.
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Timings: 8 AM to 9 PM (Mon. to Sat.) 8 AM to 6 PM ((Sunday) 24 x 7 Emergency Lab Services Available

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Visit ID	: **RCI24936**	Registration	: 09/Feb/2026 10:16AM
UHID/MR No	: RCI.0000022144	Collected	: 09/Feb/2026 10:17AM
Patient Name	: [redacted] **IPD**	Received	: 09/Feb/2026 10:17AM
Age/Gender	: 75 Y 1 M 0 D /F	Reported	: 09/Feb/2026 10:37AM
Ref Doctor	: [redacted]	Status	: Final Report
Client Name	: [redacted]	Client Code	: 315
Client Add.	:	Barcode No	: 10083177

DEPARTMENT OF HAEMATOLOGY

Test Name	Result	Unit	Bio. Ref. Range	Method
CBC				
Sample Type : WHOLE BLOOD EDTA				
HEMOGLOBIN (HB)	**10.2**	g/dl	11.0-15.0	CYANMETHEMOGLOBIN
TOTAL LEUCOCYTE COUNT (TLC)	7.34	10^3/uL	4.0-10.0	Flow cytometry
NEUTROPHIL	70	%	40-80	
LYMPHOCYTE	**16**	%	20-40	
MONOCYTE	09	%	2-10	
EOSINOPHIL	04	%	1-6	
BASOPHIL	01	%	<2	
ABSOLUTE NEUTROPHIL COUNT	5.13	10^3/uL	2.0-7.0	Automated Calculated
ABSOLUTE LYMPHOCYTE COUNT	1.17	10^3/uL	1.0-3.0	Automated Calculated
ABSOLUTE MONOCYTE COUNT	0.66	10^3/uL	0.2-1.0	Automated Calculated
ABSOLUTE EOSINOPHIL COUNT	0.29	10^3/uL	0.02-0.5	Automated Calculated
ABSOLUTE BASOPHIL COUNT	0.07	10^3/uL	0.02-0.1	Automated Calculated
RBC COUNT(RED BLOOD CELL COUNT)	3.84	million/µL	3.8-4.8	Optical Flowcytometry
PCV/HAEMATOCRIT	**30.4**	%	36-46	RBC pulse height detection
MCV	**79.2**	fL	80-100	Automated/Calculated
MCH	26.6	pg	26-34	Automated/Calculated
MCHC	33.6	g/dl	32-36	Automated/Calculated
RDW-CV	12.7	%	11.6-14.0	Automated/Calculated
RDW-SD	41.6	fL	39-46	Calculated
PCT	0.247	%	0.108-0.282	
MPV	9.6	fL	7.0-12.0	Calculated
PDW	12.1	fL	9-17	Calculated
PLATELET COUNT	257	10^3/uL	150-410	Optical Flowcytometry

[redacted]

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Consultant Pathologist

[redacted]

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IP NO: 1454

Visit ID	: **RCI24847**	Registration	: 07/Feb/2026 07:41AM
UHID/MR No	: RCI.0000022055	Collected	: 07/Feb/2026 07:52AM
Patient Name	: [redacted] **IPD**	Received	: 07/Feb/2026 07:54AM
Age/Gender	: 75 Y 1 M 0 D /F	Reported	: 07/Feb/2026 09:26AM
Ref Doctor	: [redacted]	Status	: Final Report
Client Name	: [redacted]	Client Code	: 315
Client Add	:	Barcode No	: 10083088

DEPARTMENT OF HAEMATOLOGY

Test Name	Result	Unit	Bio. Ref. Range	Method
HAEMOGLOBIN (HB)				
Sample Type : Whole Blood				
HAEMOGLOBIN (HB)	9.5	g/dl	11.0-15.0	CYANMETHEMOGLOBIN

NOTE:
Hemoglobin screening helps to diagnose conditions that affect RBCs such as anemia or polycythemia.

[redacted]

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[redacted]

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IP NO: 1454

Visit ID	: **RCI24847**	Registration	: 07/Feb/2026 07:41AM
UHID/MR No	: RCI.0000022055	Collected	: 07/Feb/2026 07:52AM
Patient Name	: [redacted] **IPD**	Received	: 07/Feb/2026 07:54AM
Age/Gender	: 75 Y 1 M 0 D /F	Reported	: 07/Feb/2026 09:26AM
Ref Doctor	: [redacted]	Status	: Final Report
Client Name	: [redacted]	Client Code	: 315
Client Add	: [redacted]	Barcode No	: 10083088

DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Range	Method
SERUM SODIUM				
Sample Type : Serum				
SERUM SODIUM	134.3	mmol/L	133-147	ISE

Increased In:
Dehydration, Vomiting or diarrhea, Burns or excessive sweating, Drugs: infusion hypertonic saline, Diabetes insipidus (DI), Post-traumatic, Renal failure and other renal causes

Decreased In:
Nephrotic syndrome, cirrhosis, pancreatitis, bowel obstruction, Blood loss, hypothyroidism, Adrenal insufficiency.

SERUM POTASSIUM				
Sample Type : Serum				
SERUM POTASSIUM	3.36	mmol/l	3.50-5.20	ISE

Increased In:
Vomiting, Diarrhea, Excessive sweating, Extensive burns, Renal failure associated with dehydration, obstruction, trauma, Renal toxicity, Addison disease, Drugs (e.g., NSAIDs, ACE inhibitors), Pseudohypoaldosteronism, use of hypertonic solutions (e.g., saline, mannitol), Intravascular hemolysis (e.g., transfusion reaction, hemolytic anemia), Rapid cellular release (e.g., crush injury, chemotherapy for leukemia or lymphoma, burns, major surgery)

Decreased In:
Uncontrolled diabetes, Renal tubular acidosis, Cushing syndrome, Malignant hypertension, Diuretics, High-dose glucocorticoids.

[redacted]

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[redacted]

[redacted]

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[redacted]

IP NO: 1454

Visit ID	: **RCI24847**	Registration	: 07/Feb/2026 07:41AM
UHID/MR No	: RCI.0000022055	Collected	: 07/Feb/2026 07:52AM
Patient Name	: [redacted] **IPD**	Received	: 07/Feb/2026 07:54AM
Age/Gender	: 75 Y 1 M 0 D /F	Reported	: 07/Feb/2026 09:26AM
Ref Doctor	: [redacted]	Status	: Final Report
Client Name	: [redacted]	Client Code	: 319
Client Add	:	Barcode No	: 10063088

DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Range	Method
SERUM CREATININE				
Sample Type : Serum				
SERUM CREATININE	0.68	mg/dL	0.5-0.9	Jaffe

Increased In:

- Diet: digestion of creatinine (meat meal), Muscle disease: gigantism, acromegaly.
- Impaired Kidney function.

Decreased In:

- Pregnancy: Normal value is 0.4-0.6 mg/dL. A value <0.4 mg/dL is abnormal and should alert the clinician to further diagnostic evaluation.
- Creatinine secretion is inhibited by certain drugs (e.g., cimetidine, trimethoprim).

[redacted]

MBBS,DCP
Consultant Pathologist

[redacted]